



Fiona Stanley Fremantle Hospitals Group

OFFICIAL

Procedure

Mental Health Crisis Centre: referral for out of hours follow up.

Reference #: FSFHG-MENH-PRO-0011

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Service Emergency Department	Medical, Nursing, Allied Health

1. Introduction

This document outlines the processes for safe and efficient management of referrals to the Mental Health Crisis Centre (MHCC) for follow up outside of business hours to ensure that:

- consumers are appropriately referred.
- referrals received, tracked and completed out of business hours.
- consumers receive appropriate follow up.
- outcomes from referrals are available to treatment teams on the next working day.

2. Terminology

Mental Health Crisis Centre (MHCC)	Multidisciplinary triage team based at Fremantle Hospital Mental Health Service operating from 08:00 to 22:00hrs seven days a week that provides the point of access for all referrals to the adult mental health service.
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Assessment & Treatment Team (ATT)	Multidisciplinary community team operating 08:30 – 16:30hrs Monday to Friday providing care coordination and short-term skills based biopsychosocial interventions for persons residing in the FSFHG catchment area for a time limited period (up to 10 weeks).
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3. Procedure

3.1. Indications for referral

- Consumers who require mental health follow up outside of normal business hours (8:00 -16:00 hrs Monday – Friday). May include individuals:
 - at increased risk
 - with mental state deterioration
 - who have been unable to be contacted during business hours.
 - awaiting admission and requiring frequent contact
 - discharged from an inpatient ward and requiring 24 hours follow up contact on a weekend
 - who have recently presented to and been discharged from Emergency Department (ED)
 - seen by Co Response and requiring follow up
- Referral to MHCC for follow up physical health issues out of hours is not appropriate.

3.2. Referral procedure

- Consumers may be referred to MHCC via e-Referral by:
 - Inpatient teams
 - ATT / Co Response
 - Community Treatment Teams (CTT)
 - Emergency Department (ED)
- Referrals from Mental Health Liaison Service – Emergency Department (MHLS-ED) may include:
 - Active FH mental health consumers requiring follow up post discharge.
 - Consumers not active with the Mental Health Service seen in ED by MHLS-ED and requiring follow up.
- Referrals from ED clinician may include:

- Consumers not assessed by the MHLS-ED with the reason for non-assessment before discharge documented in the referral.
- The MHLS-ED / ED Clinician will complete an e-Referral including:
 - as much detail as possible on the purpose of referral and risk concerns.
 - up to date patient demographics.
 - a follow up with call to MHCC when able.

MHCC will access relevant details from the medical record as required.

Prior notice to MHCC via email or face-to-face will suffice for active consumers of ATT or CTT who:

- who are expected to attend after-hours for a depot.
- require completion of monitoring post olanzapine depot

An e-referral is not required.

- MHCC maintains an electronic contact log documenting all contact with active consumers and will feedback to the ATT and CTTs at the end of each shift.
- MHCC will refer patients to the Aboriginal Mental Health Liaison Service where appropriate.
- The outcome of each MHCC contact will be documented in:
 - the consumer's medical record
 - as a service event in PSOLIS, including contacts which are attempted but not successful.
- Where contact is not successful and there are clinical concerns, MHCC staff will discuss further action with the ATT Duty Officer/ Hospital Out of Hours (HOOT) Manager or Duty Medical Officer. If appropriate the Consultant Psychiatrist on call will be consulted.

4. Compliance/performance monitoring

The MHCC Team Leader will monitor compliance with this document via routine clinical incident review processes.

5. Related policy documents

- SMHS:
 - [Community/Home Visiting](#) (SMHS: 131)
- FSFHG:

- Clinical handover (FSFH-HW-POL-0035)
- Clinical documentation (FSFH-HW-POL-0039)
- Mental Health patient management: Emergency Department (FSFH-HW-POL-0052)
- Clinical risk assessment and management (FSFH-HW-POL-0017)
- Discharge follow up for mental health inpatients (FSH-MENH-POL-0019)
- Did not attend: Mental Health outpatient appointments (FSFH-MENH-PRO-0001)
- Community mental health status assessment (FSFH-MENH-PRO-0006)

6. Related standards

NSQHS Standards:

- Comprehensive Care
- Communicating for Safety
- Recognising and Responding to Acute Deterioration

Chief Psychiatrists Standards for Clinical Care

- Assessment
- Risk assessment and management.

7. Authorisation

EXECUTIVE SPONSOR: Nurse Director, Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	06/2007			FH Committees	06/2009
2	2009			FH Committees	2012
3	04/2016	ATT Coordinator	FHMHS Clinical Governance Committee	FHHS Policy Committee	03/2020
4	03/2021	ATT Team Leader		FSFHG Policy Committee	03/2025
5	05/2025	MHCC Team Leader	MH Clinical Governance Sub Committee NMPGC	MH Clinical Governance Sub Committee	05/2028
6	06/2055	Policy Coordinator, SMHS MH	Minor update, no change to review date		05/2028